KinPath: Revised Year-One Strategy and Market-Entry Plan

Working name: KinPath, subject to trademark clearance and domain availability
Launch market: Atlanta, Georgia
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Executive summary

KinPath should not begin as a broad funeral marketplace, a national death-doula network, or a full next-of-kin operating system. Those are credible long-term expansion opportunities, but pursuing them all at once would require too much capital, introduce unnecessary regulatory and operating risk, and make it difficult to prove a repeatable customer-acquisition model.

KinPath should begin as a bereavement-operations and family-guidance platform for hospices.

The first product helps hospice teams manage bereavement outreach, documentation, and follow-up while giving families a clear, compassionate handoff after a death. The aim is to turn fragmented manual work—spreadsheets, phone lists, shared inboxes, calendars, and informal notes—into a reliable, auditable workflow.

The initial thesis is:

> KinPath helps hospice teams manage bereavement follow-up and gives families a guided plan for what comes next.

The hospice wedge is attractive because a hospice must maintain an organized bereavement program and make bereavement services available to the patient's family and caregivers for up to one year following death. KinPath's purpose is not to replace hospice clinical, spiritual, social-work, legal, or funeral professionals. It is to make the nonclinical operational workflow more organized, visible, consistent, and easier to document.

The year-one objective is **evidence, not feature volume.** By the end of year one, KinPath should have a validated workflow, live design partners or customers, measurable operational outcomes, a repeatable implementation approach, and sufficient proof to support a larger institutional-ready pre-seed or seed round.

1. Positioning and category

What KinPath is now

KinPath is a **bereavement-operations and family-guidance platform** for hospice organizations.

For hospice teams, it provides:

- A case-based workflow for post-death bereavement follow-up
- Configurable outreach schedules and due-date visibility
- Contact logging, task tracking, and handoff records
- Staff visibility into outstanding work and next actions
- Reporting that helps organize activity records for internal review and survey preparation
- A secure, family-directed handoff to practical next-step guidance

For families, it provides:

- A calm first-72-hours experience
- Clear separation of what needs attention now and what can wait
- A task plan, document-request tracking, and approved support resources
- A way to invite and coordinate trusted helpers at a limited, permissioned level
- Clear boundaries around what KinPath does not do

What KinPath is not in year one

KinPath is not, initially:

- A funeral home, funeral arranger, or party that negotiates or books funeral services
- A legal service, estate-planning provider, probate advisor, or claims representative
- A licensed clinical counseling provider
- An insurer claims-processing system
- A national marketplace for death doulas or vendors
- A full pre-need planning or will-drafting platform
- A replacement for hospice staff, an EHR, an EAP, a funeral director, a lawyer, or a therapist

Long-term category vision

The long-term opportunity remains broader: KinPath can become a trusted next-of-kin coordination layer across hospice, employers, hospitals, insurers, senior living, and families.

But that vision should be earned in stages:

1. Win hospice bereavement operations
2. Prove family adoption and administrative value

3. Add benefits discovery and document-readiness workflows
4. Expand to adjacent care settings and partner channels
5. Add carefully governed service networks and integrations only after product-market fit

2. The initial customer and workflow

First ideal customer profile

The first ideal customer is a Georgia Medicare-certified hospice organization with:

- Approximately 250–1,000 annual deaths or a sufficiently large bereavement caseload
- More than one location, or enough volume that manual tracking is difficult
- A designated bereavement coordinator, social-work leader, chaplain, or similar workflow owner
- Current use of spreadsheets, calendars, shared inboxes, call lists, paper records, or disconnected systems
- An executive sponsor who cares about staff efficiency, reliable follow-up, family experience, or readiness for reviews and surveys
- Willingness to participate as a design partner and provide de-identified workflow feedback

First end user

The principal daily user is likely to be a bereavement coordinator, social worker, chaplain, or administrator. The product must save this person time and reduce cognitive burden. If the workflow does not make that person's day easier, it will not be adopted consistently.

First family user

The family user is a surviving relative, caregiver, chosen-family member, or trusted helper. The product must not assume that the account holder is the legal next of kin, executor, beneficiary, or legally authorized decision-maker.

Use neutral product language such as **workspace administrator** or **family coordinator**, not language that implies legal authority.

Core workflow to solve

The initial workflow begins at notification of death and includes:

1. Staff creates or activates a bereavement case
2. The care team records the designated family contact and appropriate outreach preferences
3. KinPath applies the hospice's selected outreach cadence
4. Staff sees due, upcoming, completed, and overdue outreach
5. Staff records contact attempts, outcomes, follow-up needs, and next actions

6. The family receives an optional secure invitation to the KinPath family experience
7. The family sees practical immediate next steps and support resources
8. Staff can request, track, or hand off limited practical tasks where appropriate
9. Hospice leadership can review activity and generate a structured operational report

3. Revised MVP scope

Build first: months 1–5

Build a **secure responsive web application**, not separate native iOS and Android apps. A mobile-friendly web link is faster, less expensive, easier to update, and adequate for testing family activation.

Hospice staff workflow

- Staff sign-in with multi-factor authentication for administrative accounts
- Case list with assigned owner, status, upcoming outreach, overdue work, and priority flags
- Configurable outreach cadence based on the hospice's workflow
- Contact log with date, channel, outcome, follow-up action, and next contact date
- Task list and case timeline
- Basic role-based access
- Staff-facing notes only where permitted by the organization's policy and data-flow design
- Exportable activity report designed to support internal review and survey preparation

Family handoff

- Secure family invitation by email, text, or a shareable controlled link
- First-72-hours guide with "do now," "can wait," and "who can help" sections
- Practical task list and light delegation to trusted helpers
- Document-request and document-receipt tracking
- Clearly sourced general information and official-resource links
- Crisis and emergency-support routing appropriate to the United States
- Accessibility-first design: plain language, readable typography, large-text support, and a multilingual-ready structure

Trust foundation

- Encryption in transit and at rest
- Role-based and least-privilege access
- Audit logging for staff access and key case activity

- Secure backups and recovery process
- Data-minimization rules
- Retention and deletion policy
- Vendor inventory and security review process
- Written incident-response checklist
- Clear user terms, privacy notice, and product-boundary statements

Do not build yet

Defer these until KinPath demonstrates adoption, operational value, and a repeatable hospice implementation process:

- Native iOS and Android apps
- Marketplace booking, payment flows, featured listings, commissions, referral fees, or vendor ranking
- National death-doula recruitment or matching
- Automated benefits eligibility determinations
- Filing claims on behalf of families
- Full Benefits Recovery Engine integrations
- Detailed expense splitting or family-voting mechanics
- Hospital decedent-affairs product
- Senior-living offering
- Employer dashboard and PEPM sales motion
- HRIS, EAP, insurer, claims-system, or EHR integrations
- Pre-need planning and estate document storage beyond the narrow family workflow
- Will drafting, probate guidance, or legal service delivery
- Licensed grief counseling services
- National expansion to additional metros

4. Family experience and safety design

First-72-hours experience

The first family interaction must lower stress, not create a long intake form or force an overwhelming checklist.

The initial screen should ask only enough to tailor the immediate guide. It should distinguish between expected hospice death, hospital death, sudden death, death at home, and other pathways only when that distinction meaningfully changes guidance.

The family experience should include:

- **Do now:** a small number of time-sensitive steps
- **Can wait:** nonurgent administrative items that should not be forced on a grieving person immediately
- **Who can help:** hospice team, funeral director, faith leader, employer/HR, attorney, benefits office, or trusted relative, as relevant
- **Pause and return:** a visible option to defer the workflow without pressure
- **Share with a helper:** limited invitations and task delegation
- **Get support:** grief resources and crisis-routing information

Family permissions and disputes

The product must not create an appearance that KinPath determines authority over remains, finances, estate decisions, or family disagreements.

Use these roles:

Role	What the product permits	What it does not establish
Workspace administrator	Manages workspace settings, invitations, and task assignments	Legal authority, executor status, next-of-kin status, or power to make binding decisions
Family collaborator	Sees specifically permitted tasks and information; can complete tasks	Automatic access to all documents or financial information
Read-only supporter	Sees selected updates or event information	Access to sensitive case data unless explicitly granted
Professional helper	Participates only in consented workflow areas	Legal, clinical, funeral, or financial authority unless separately qualified and contracted

Maintain an audit record of invitations, permissions, access changes, and corrections. Preserve the history of changes but allow factual errors to be corrected transparently.

For disagreement-sensitive matters, KinPath should provide neutral information, record that a question needs resolution, and direct users to the appropriate authorized decision-maker or qualified professional. KinPath should not adjudicate disputes.

Crisis and support routing

KinPath should include a clear support route for users who express or indicate immediate emotional crisis. For U.S. users, make 988 and local emergency options easy to find. Do not present the app as emergency care or clinical treatment.

5. Benefits discovery: revised approach

The benefits concept remains strategically valuable, but the initial approach must be cautious and evidence-based.

Phase-one name and scope

Call the early feature **Benefits Discovery and Claims Readiness**, not “Benefits Recovery Engine.”

It should:

- Ask a small set of screening questions about employment, military service, insurance, retirement plans, unions, and other potential sources
- Identify benefits that may be worth checking
- Link to official sources and present plain-language document checklists
- Track family-reported status: not started, gathering documents, submitted, awaiting response, paid, denied, or no longer pursuing
- Help organize a claim-ready document bundle

It should not:

- State that a user is eligible for a specific benefit
- Promise a financial outcome
- Treat potential value as recovered value
- Provide individualized legal, tax, or financial advice
- File claims on a family’s behalf without appropriate authority, partner agreements, and legal review

Measurement rules

Report three different values, never one blended number:

- **Potential value identified:** benefits a family may wish to investigate
- **Claims submitted:** claims that the family reports submitting
- **Verified payments received:** funds confirmed as paid to the family

Only verified payments should be called “recovered.”

6. Compliance and operating guardrails

Professional-service boundaries

Before KinPath coordinates vendors, charges a coordination fee, ranks providers, accepts referral compensation, arranges services, or quotes funeral prices, obtain state-specific legal advice. These activities can create licensing, consumer-protection, fee-splitting, or conflict-of-interest issues.

The year-one product should stay on the safer side of the line:

- Provide neutral directories and general information only where reviewed
- Make any commercial relationship conspicuous to the family

- Do not arrange at-need services
- Do not negotiate vendor prices
- Do not represent that KinPath or a coordinator is acting as a funeral director
- Do not provide legal advice or individualized probate/estate direction

Privacy and data-flow modes

Treat the following as separate deployment modes requiring separate review:

Mode	Description	Practical requirement
Consumer-directed	Family independently creates an account and enters information	Clear privacy notice, consent, and data-minimization practices
Partner-sponsored referral	Hospice gives a family a link or QR code but does not transmit case data	Define referral process and confirm no inappropriate disclosure occurs
Integrated partner workflow	Hospice transmits data to KinPath or views detailed case-level information	Formal data-flow, security, contracting, and potential BAA review before launch
Clinical-care expansion	KinPath delivers or coordinates clinical services	Additional licensing, clinical governance, emergency, privacy, and recordkeeping requirements

Do not make a general claim that KinPath is outside HIPAA based solely on a consumer download model. The legal posture depends on actual data flows, services, contracts, and relationships.

Security and governance

Technical encryption is necessary but is not a complete security program. Establish these practices from the first production build:

- Named person responsible for security and privacy decisions
- Data inventory and classification
- Least-privilege access controls
- Multi-factor authentication for staff and administrators
- Quarterly access review process
- Audit logging and monitoring
- Secure development practices
- Vendor-risk review for cloud, messaging, analytics, document storage, payment, telehealth, and AI providers
- Written incident response and breach-notification decision tree
- Retention and secure deletion schedule
- No sale of personal information and no behavioral advertising using sensitive family data
- Penetration testing before broad institutional rollout

SOC 2 should be treated as a staged procurement investment. Build controls and evidence discipline early; undertake a formal audit when customer requirements, enterprise contracting, or scale justify it.

7. Design-partner and pilot strategy

Customer-discovery goals

Before a production build, conduct 25–30 interviews:

- 10 hospice bereavement coordinators, social workers, or chaplains
- 5 hospice executives or administrators
- 5 recently bereaved family members, recruited ethically through trusted channels
- 5 death doulas or care navigators
- 5 optional HR/benefits leaders for future-market insight only

Also complete five workflow-shadowing or de-identified workflow-mapping sessions.

Design partner target

Secure 3–5 Atlanta hospice design partners. Each should provide:

- Executive sponsor
- Frontline workflow champion
- Structured feedback sessions
- De-identified baseline workflow measures
- Prototype review
- Written letter of intent or pilot agreement
- A defined conversion discussion after the pilot

KinPath should offer founding-partner terms, high-touch implementation, and a tightly scoped pilot—not unlimited free customization.

Pilot success scorecard

Area	Baseline	Pilot evidence to seek
Staff burden	Minutes per case, manual list maintenance, report-preparation effort	Fewer manual steps or measured reduction in avoidable work
Outreach reliability	Due/overdue contacts, missed follow-up, visibility of next actions	Improved ability to complete and document the selected cadence
Family activation	Current referral/handoff method and response rate	Meaningful share of invited families reaches a first useful action
Family usefulness	Repeated questions and current feedback	Families report more clarity about next steps

Area	Baseline	Pilot evidence to seek
Implementation	Staff training and workflow constraints	Launch occurs in a defined timeframe without excessive burden
Commercial fit	Current budget and procurement constraints	Partner engages in a paid-conversion decision after review

All numerical ROI claims are hypotheses until measured in actual pilots. Do not market speculative productivity, labor-reduction, claims-cost, or clinical-outcome percentages as established results.

8. Revised 12-month operating plan

Months 1–2: Discovery, legal foundation, and prototype

Primary goal: validate the buyer problem and earn the right to build.

- Write the one-page product brief and first-workflow definition
- Build a list of 40 Atlanta hospice prospects
- Conduct at least 15 discovery interviews
- Begin five workflow-mapping sessions
- Build a clickable prototype, not production software
- Obtain limited-scope legal guidance on entity formation, trademark, professional boundaries, privacy/data flows, and pilot contracting
- Create basic landing page, CRM, interview-notes process, and founder deck
- Apply to relevant Atlanta health-tech/startup programs

Exit criteria: repeated workflow pain, prototype feedback from real users, at least one committed co-design conversation, and a clear MVP hypothesis.

Months 3–4: Design partners and pilot preparation

Primary goal: turn research into commitments.

- Reach 25–30 total interviews
- Secure 3–5 written design-partner LOIs or pilot commitments
- Agree on baseline metrics and pilot scorecard
- Finalize core legal documents and data-flow approach
- Select a technical lead or builder through a fixed, milestone-based scope
- Build only the responsive web MVP required for the pilot
- Develop staff onboarding, support, and weekly pilot-review procedures

Exit criteria: signed pilot plans, named sponsor and champion at each partner, defined implementation sequence, and a functional narrow MVP.

Months 5–7: Controlled pilot launch

Primary goal: demonstrate use and operational value.

- Launch with 2–3 hospice partners using a limited cohort rather than all eligible cases at once
- Configure each organization's outreach cadence
- Train a limited set of staff users
- Measure baseline and ongoing workflow burden, outreach reliability, and family activation
- Conduct weekly implementation reviews
- Fix adoption blockers before adding features
- Gather family feedback with ethical, optional, and privacy-respecting methods

Exit criteria: consistent staff usage, documented workflow improvement, initial family value signals, and at least one partner entering paid-conversion discussion.

Months 8–10: Prove repeatability and monetize

Primary goal: convert pilots and standardize implementation.

- Convert successful partners to paid annual contracts
- Create a standard onboarding plan, implementation guide, training program, and customer-success cadence
- Build a procurement-ready trust packet: security overview, privacy posture, subcontractor list, and standard contractual materials
- Publish an approved anonymized case study or outcome brief
- Add only features that address measured adoption bottlenecks
- Test the limited Benefits Discovery and Claims Readiness workflow with appropriate disclaimers and review

Exit criteria: 3–5 paying hospice customers or a comparable concentration of contracted recurring revenue, a repeatable onboarding process, and evidence of renewal intent.

Months 11–12: Prepare responsible expansion

Primary goal: determine what should scale next.

- Review retention, implementation time, support cost, gross margin, and customer feedback
- Identify whether the next expansion is more hospice accounts, a second Georgia region, benefits discovery, senior living, or a carefully selected employer referral pilot
- Do not launch multiple new verticals simultaneously
- Prepare a fundraising process only if the results demonstrate repeatability
- Build a year-one evidence report focused on real operating metrics—not broad claims of market dominance

Exit criteria: clear product-market evidence, a defensible use-of-funds plan, and a narrow expansion thesis.

9. Capital plan

Stage 1: Validation capital

Target: $5,000–$25,000

Use this for:

- Customer discovery
- Prototype creation
- Limited legal and trademark consultation
- Landing page, CRM, basic branding, and outreach
- Travel and meetings
- Design-partner materials

Do not use validation capital to commission a full consumer app or complex marketplace.

Stage 2: Pilot-preparation capital

Target: $50,000–$150,000

Use this for:

- Narrow responsive web MVP
- Fractional technical leadership or a vetted development partner
- Initial privacy/security controls
- Core legal documents and data-flow review
- Pilot onboarding and customer support
- Two to three controlled hospice pilots

Potential sources include customer-funded pilots, a carefully structured friends-and-family round, angels, accelerators, pitch prizes, and qualified non-dilutive programs.

Stage 3: Institutional-ready pre-seed

Target: $450,000–$650,000

Raise only when KinPath has design-partner proof and early live-use evidence.

Use this for:

Use of funds	Planning range
Product and engineering	$190,000–$235,000

Use of funds	Planning range
Founder and lean operations	$75,000–$105,000
Legal, privacy, compliance, and insurance	$55,000–$85,000
Security and infrastructure	$30,000–$50,000
Pilot onboarding and customer success	$45,000–$65,000
Market discovery and sales	$25,000–$40,000
Contingency	$40,000–$60,000
Total	$460,000–$640,000

Target a monthly burn that provides 12–15 months of runway. Avoid fixed office costs unless they directly support customer work or team productivity.

10. Commercial model

Initial pricing approach

Do not set a permanent public price before testing willingness to pay and support cost.

Test one of these structures with design partners:

- Annual platform fee based on annual death volume
- Core hospice workflow subscription plus a per-family activation fee
- Multi-site annual agreement with implementation and premium-support fees

Use paid pilots wherever feasible. Even a modest paid pilot is stronger validation than free interest.

Initial commercial target

The first commercial objective is not 20–30 employers, multiple hospital systems, a second metro, or enterprise insurer contracts. It is:

- 3–5 paying hospice customers
- Measurable usage and workflow value
- A standard implementation playbook
- Strong evidence of renewal intent
- A credible unit-economic model

Required business metrics

Track:

- Sales-cycle length
- Number of discovery calls to design-partner commitment
- Pilot-to-paid conversion
- Annual contract value
- Implementation time
- Staff adoption
- Family activation and first meaningful action
- Support cost per customer and per activated family
- Gross margin
- Customer acquisition cost
- Churn and renewal intent
- Revenue concentration
- Net revenue retention once expansions exist

For benefits discovery, separately track potential sources identified, claims started, claims submitted, and verified payments received.

11. Brand recommendation

KinPath remains the recommended working name because it is warm, short, family-oriented, and suggests guided progress without using morbid or overly clinical language.

Before committing:

- Conduct professional trademark clearance for relevant categories and confusingly similar marks
- Check domain availability and defensible alternatives
- Test the name and tagline with bereaved families, hospice frontline staff, and institutional buyers
- Prepare fallback options such as AfterGuide or Kindred Path only after equivalent trademark review

Use a clearer external tagline than “next-of-kin operating system.” That phrase can remain useful internally and in investor materials, but it may feel abstract to families and frontline hospice teams.

Recommended public-facing language:

> KinPath helps families manage what comes next—and gives care organizations a reliable way to support them.

Recommended hospice-facing language:

> A bereavement-operations and family-guidance platform for the year after loss.

12. Final decision rules

Do not move to the next investment stage simply because time has passed. Move when evidence supports it.

Continue with the hospice wedge when:

- Multiple hospice teams describe the same urgent workflow problem
- Staff engage with the prototype and agree it fits their process
- At least three organizations make written design-partner commitments
- Pilot participants accept clear measurement and a paid-conversion discussion
- The product reduces friction without creating unacceptable privacy, security, legal, or implementation burden

Pause, narrow, or revise the approach when:

- The problem is not repeated across target customers
- Partners will not provide a champion, workflow access, or pilot commitment
- Family activation is consistently low despite a simple and compassionate handoff
- The product adds more implementation burden than it removes
- Legal or privacy requirements materially change the viable deployment model

The company should earn the right to become a broad next-of-kin platform. Its first job is much simpler: make one hospice bereavement workflow clearly better for staff and families, prove it with evidence, and build from there.